



IMDELLTRA[®]
(tarlatamab-dlle)



Extensive-Stage Small Cell Lung Cancer

**with disease progression on or after platinum-based chemotherapy*

Mechanism of Action:

IMDELLTRA[®] (Tarlatamab-dlle) is a bispecific T-cell engager (BiTE) therapy that helps the immune system recognize and attack small cell lung cancer (SCLC) cells. It works by binding to delta-like ligand 3 (DLL3), a protein that is normally found inside healthy cells but is abnormally present on the surface of SCLC cells. This abnormal surface expression makes DLL3 an important target in SCLC.

The drug is built with two single-chain variable fragment (scFv) binding domains joined by a flexible linker. One domain attaches to DLL3 on tumor cells, while the other binds to CD3 receptors on T cells. This connection brings T cells into direct contact with DLL3-positive cancer cells, leading to T-cell activation, release of inflammatory cytokines, and subsequent killing (lysis) of the tumor cells.

Pre- & Post-Infusion

Labs:

Pre-Infusion only: hCG

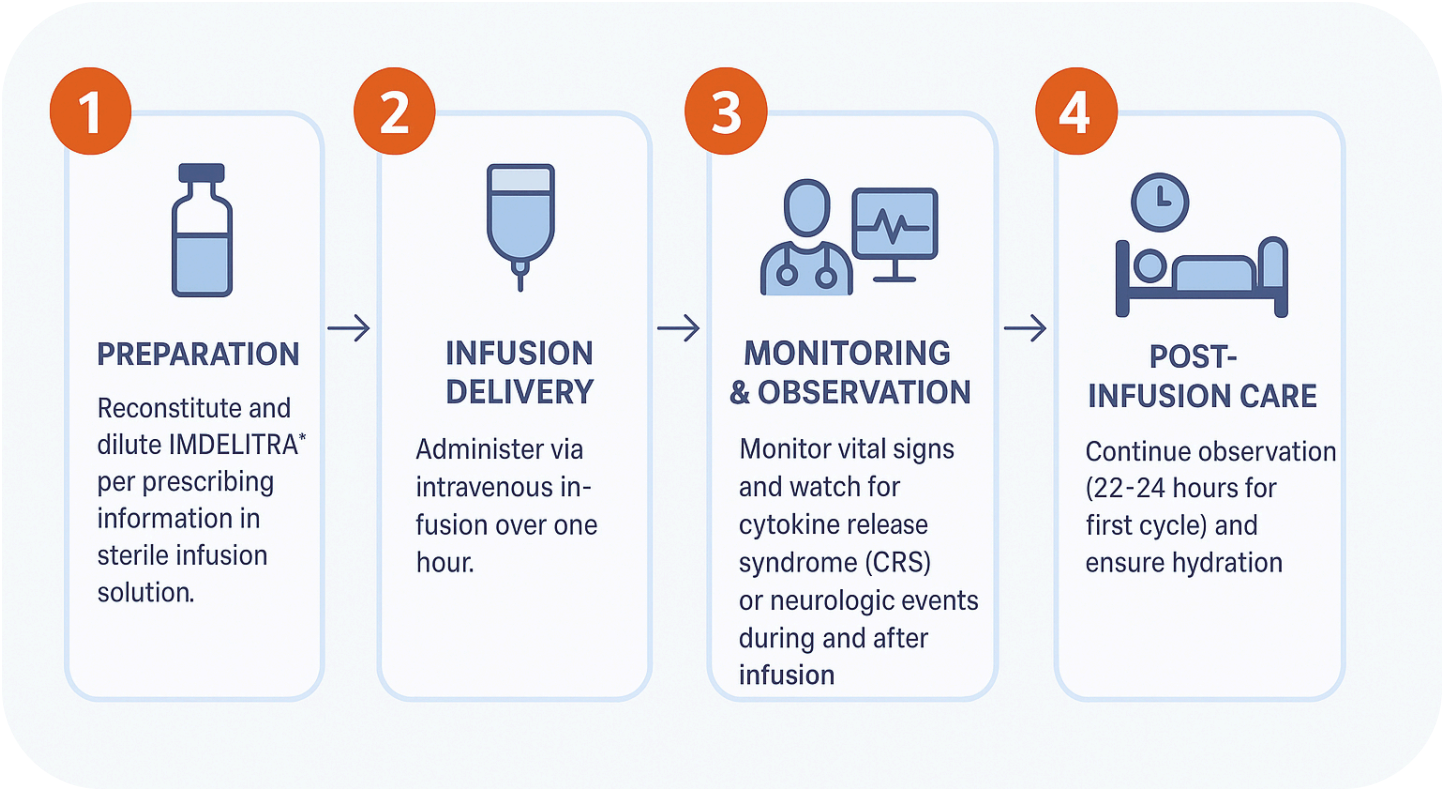
Pre-Infusion and prior to each dose: CBC, ALT/AST, Bilirubin

Other affected levels: lymphocytes (L), sodium (L), neutrophils (L), hemoglobin (L), potassium (L), white blood cells (L), platelets (L), alanine aminotransferase (L), uric acid (H), activated partial thromboplastin time (H), aspartate aminotransferase (H)

Pre-Medications: *reduces risk of cytokine release syndrome*

Dexamethasone 8 mg IV within 1 hour prior to infusion; Day 1 and Day 8

Administration



The IV catheter for concomitant medications administration can be used to administer the IMDELLTRA® infusion. To ensure patency, flush the IV catheter over 3-5 mins using 0.9% Sodium Chloride for Injection. Administer the reconstituted and diluted IMDELLTRA® as an IV infusion at a constant flow rate of 250 mL/hour for 1 hour using an infusion pump. The pump should be programmable, lockable, non-elastomeric, and have an alarm.



Adverse Reactions

Cytokine release syndrome (CRS)

Fatigue

Pyrexia

Dysgeusia

Decreased appetite

Musculoskeletal pain

Constipation

Anemia

Warnings

Neurologic Toxicity; ICANS

Cytopenias

Infections

Hepatotoxicity

Hypersensitivity

Embryo-Fetal Toxicity

Monitoring

Cycle 1, Day 1 and Day 8	Monitor patients from the start of the infusion for 22-24 hours on Cycle 1, Day 1 and Cycle 1, Day 8 in a healthcare setting. Recommend that patients remain within 1-hour of an appropriate healthcare setting for a total of 48 hours from start of the infusion, accompanied by a caregiver.
Cycle 1, Day 15	Observe patients for 6-8 hours post-infusion ^b .
Cycle 2, Day 1 and Day 15	Observe patients for 6-8 hours post-infusion ^b .
Cycles 3 and 4, Day 1 and Day 15	Observe patients for 3-4 hours post-infusion ^b .
Cycle 5 and subsequent infusions, Day 1 and Day 15	Observe patients for 2 hours post-infusion ^b .

a= Administer recommended concomitant medications before and after Cycle 1 IMDELLTRA[®] infusions as described.

b= Extended monitoring in a healthcare setting is not required unless the patient experiences Grade ≥ 2 CRS, ICANS or neurological toxicity during prior treatments. See *Monitoring* below.

Infusion Management for Cytokine Release Syndrome (CRS)

Grade 1 (e.g., fever $\geq 100.4^{\circ}\text{F}$ without hypotension or hypoxia)	Withhold IMDELLTRA[®] until event resolves, then resume IMDELLTRA[®] at the next scheduled dose^b. Administer symptomatic treatment (e.g., acetaminophen) for fever.
Grade 2 (e.g., Fever $\geq 100.4^{\circ}\text{F}$, hypotension responsive to fluids not requiring, vasopressors and/or hypoxia requiring low flow nasal cannula or blow-by)	Withhold IMDELLTRA[®] until event resolves, then resume IMDELLTRA[®] at the next scheduled dose^b. - Recommend hospitalization for a minimum of 24 hours with cardiac telemetry and pulse oximetry - Administer symptomatic treatment (e.g., acetaminophen) for fever - Administer supplemental oxygen and intravenous fluids when indicated - Consider dexamethasone^c (or equivalent) 8 mg IV. - Consider tocilizumab (or equivalent) When resuming treatment at the next planned dose, monitor patients from the start of the IMDELLTRA[®] infusion for 22 to 24 hours in an appropriate healthcare setting.
Grade 3 (e.g., 38°C with: Hemodynamic instability requiring a vasopressor (with or without vasopressin) or worsening hypoxia or respiratory distress requiring high flow nasal canula ($> 6 \text{ L/min}$ oxygen) or face mask)	Withhold IMDELLTRA[®] until the event resolves, then resume IMDELLTRA[®] at the next scheduled dose^b. For recurrent Grade 3 events, permanently discontinue IMDELLTRA[®]. In addition to Grade 2 treatment: - Recommend intensive monitoring, e.g., - ICU care - Administer dexamethasone^c (or equivalent) 8 mg IV every 8 hours up to 3 doses - Vasopressor support as needed - High flow oxygen support as needed - Recommend tocilizumab (or equivalent) - Prior to the next dose, administer concomitant medications as recommended for Cycle 1 When resuming treatment at the next planned dose, monitor patients from the start of the IMDELLTRA[®] infusion for 22 to 24 hours in an appropriate healthcare setting.
Grade 4 (e.g., Fever $\geq 100.4^{\circ}\text{F}$, Hemodynamic	Permanently discontinue IMDELLTRA[®]. - ICU care - Per Grade 3 treatment

instability requiring multiple vasopressors (excluding vasopressin), worsening hypoxia or respiratory distress despite oxygen administration requiring positive pressure)

- Recommend tocilizumab (or equivalent)
- Perform laboratory testing to monitor for disseminated intravascular coagulation (DIC) and hematology parameters
- Perform laboratory testing to monitor for pulmonary, cardiac, renal, and hepatic function

a= CRS based on American Society for Transplantation and Cellular Therapy (ASTCT) Consensus Grading

b= See Table 4 in PI for recommendations on restarting IMDELLTRA® after dose delays

c= Taper steroids per standard of care guidelines.

Infusion Management for Neurologic Toxicity; Immune Effector Cell- Associated Neurotoxicity Syndrome (ICANS)

Grade 1^a ICE score 7-9^b with no depressed level of consciousness.	Withhold IMDELLTRA[®] until ICANS resolves, then resume IMDELLTRA[®] at the next scheduled dose^c. • Supportive care
Grade 2^a ICE score 3-6^b and/or mild somnolence awaking to voice.	Withhold IMDELLTRA[®] until ICANS resolves, then resume IMDELLTRA[®] at the next scheduled dose^c. • Supportive care • Dexamethasone^d (or equivalent) 10 mg IV. Can repeat every 6 hours or methylprednisolone 1 mg/kg IV every 12 hours if symptoms worsen • Monitor neurologic symptoms and consider consultation with neurologist and other specialists for further evaluation and management • Monitor patients for 22 to 24 hours following the next dose of IMDELLTRA[®]
Grade 3^a ICE score 0-2^b and/or depressed level of consciousness awakening only to tactile stimulus and/or any clinical seizure focal or generalized that resolves rapidly or nonconvulsive seizures on EEG that resolve with intervention and/or Focal or local edema on neuroimaging.	Withhold IMDELLTRA[®] until the ICANS resolves, then resume IMDELLTRA[®] at the next scheduled dose^c. If there is no improvement to grade ≤ 1 within 7 days or grade 3 toxicity reoccurs within 7 days of reinitiation, permanently discontinue IMDELLTRA[®]. For recurrent grade 3 events, permanently discontinue. • Recommend intensive monitoring, e.g., ICU care • Consider mechanical ventilation for airway protection • Dexamethasone^d (or equivalent) 10 mg IV every 6 hours or methylprednisolone 1 mg/kg IV every 12 hours • Consider repeat neuroimaging (CT or MRI) every 2-3 days if patient has persistent Grade ≥ 3 neurotoxicity • Monitor patients for 22 to 24 hours following the next dose of IMDELLTRA[®]
Grade 4^a ICE score 0^b (patient is unarousable and unable to perform ICE) and/or Stupor or coma and/or Life-threatening prolonged seizure (>5 minutes) or repetitive clinical or electrical seizures without return to baseline in between	Permanently discontinue IMDELLTRA[®]. • ICU care • Consider mechanical ventilation for airway protection • High dose corticosteroids^d • Consider repeat neuroimaging (CT or MRI) every 2-3 days if patient has persistent Grade ≥ 3 neurotoxicity • Treat convulsive status epilepticus per institutional guidelines

and/or diffuse cerebral edema on neuroimaging, decerebrate or decorticate posturing or papilledema, cranial nerve VI palsy, or Cushing's triad.

a= ICANS based on American Society for Transplantation and Cellular Therapy (ASTCT) Consensus Grading

b= If patient is arousable and able to perform Immune Effector Cell-Associated Encephalopathy (ICE) Assessment, assess: Orientation (oriented to year, month, city, hospital = 4 points); Naming (names 3 objects, e.g., point to clock, pen, button = 3 points); Following commands (e.g., "show me 2 fingers" or "close your eyes and stick out your tongue" = 1 point); Writing (ability to write a standard sentence = 1 point); and Attention (count backwards from 100 by ten = 1 point). If patient is unarousable and unable to perform ICE Assessment (Grade 4 ICANS) = 0 points.

c= See Table 4 in PI for recommendations on restarting IMDELLTRA® after dose delays

d= Taper steroids per standard of care guidelines

Patient Counseling

- Monitoring requirements post-infusion
- Frequent laboratory testing

Patient-specific Instructions:

- Advise patients who experience neurologic toxicity or symptoms of ICANS to refrain from driving or operating heavy or potentially dangerous machinery and engaging in hazardous occupations or activities during treatment with IMDELLTRA®.
- Advise pregnant women and females of reproductive potential of the potential risk to a fetus.
- Advise females of reproductive potential to inform their healthcare provider if they are pregnant or become pregnant.
- Advise females of reproductive potential to use effective contraception during treatment with IMDELLTRA® and for 2 months after the last dose.
- Advise women not to breastfeed during treatment with IMDELLTRA® and for 2 months after the last dose.

Contact their healthcare provider regarding signs & symptoms of the following:

- Cytokine release syndrome (CRS): pyrexia, hypotension, fatigue, tachycardia, headache, hypoxia, nausea and vomiting
- Immune Effector Cell-Associated Neurotoxicity Syndrome (ICANS): encephalopathy, confusion, delirium, seizure, ataxia, weakness or numbness of arms and legs, tremor, and headache
- Cytopenias: neutropenia and febrile neutropenia, anemia, and thrombocytopenia
- Infections
- Hepatotoxicity
- Hypersensitivity reactions
- Embryo-Fetal Toxicity



IMDELLTRA® Package Insert (PI) Link

www.imdelltrahcp.com
